

At TPT initiation, decide patient category

- If never had TPT before or took TPT < 4 weeks, document as **new**.
- If completed TPT before or took TPT ≥ 4 weeks and stopped (due to adverse event, developed TB or was lost to follow up), document as **previously treated**.

Treat the patient needing TPT according to chosen regimen and weight

- Give **pyridoxine** together with TPT.
- If severe peripheral neuropathy, active liver disease or known alcohol use disorder, defer TPT.

Rifapentine and isoniazid (3HP):

- 3HP is weekly rifapentine and isoniazid for 3 months.
- Give with or immediately after eating.
- Rifapentine decreases levels of protease inhibitors (lopinavir/atazanavir/ritonavir), nevirapine, and dolutegravir (when starting): use instead **12H**.
- Rifapentine decreases levels of oral contraceptive and subdermal implant: use instead barrier method *and* injectable or IUD contraceptive.

Weight (kg)	Isoniazid (weekly)	Rifapentine (weekly)
	300mg tablets (weekly)	150mg tablets (weekly)
25 – 29.9	2 tablets	4 tablets
≥ 30	3 tablets	6 tablets

Isoniazid (6H and 12H):

- 6H is daily isoniazid for 6 months.
- 12H is daily isoniazid for 12 months.

Weight (kg)	Isoniazid (Daily)	
	100mg tablet (daily)	300mg tablet (daily)
2 – 3.4	¼ tablet	-
3.5 – 4.9	½ tablet	-
5 – 7.4	¾ tablet	-
7.5 – 9.9	1 tablet	-
10 – 14.9	1 ½ tablet	-
15 – 19.9	2 tablets	-
≥ 20	3 tablets	1 tablet

Rifampicin and isoniazid (3RH):

- 3RH is daily dosing rifampicin and isoniazid for 3 months.
- Rifampicin interacts with ART²: adjust doses or TPT regimen.

Weight (kg)	RH (Daily)		
	300/150	150/75	75/50 (use mL if tablet dispersed in water)
2-2.9	-	-	½ tablet 5mL
3-3.9	-	-	¾ tablet 7.5mL
4-5.9	-	-	1 tablet 10mL
6-7.9	-	-	1 ½ tablet 15mL
8-11.9	-	-	2 tablets 20mL
12-15.9	-	-	3 tablets 30mL
16-24.9	-	-	4 tablets 40mL
25 – 37.9	-	2 tablets	-
38 – 54.9	-	3 tablets	-
≥ 55	2 tablets	-	-

Rifampicin (4R):

- 4R is daily dosing rifampicin for 4 months.
- Rifampicin interacts with ART²: adjust doses or TPT regimen.

< 10 years	15mg/kg daily
≥ 10 years	10mg/kg daily

Pyridoxine

- Give pyridoxine whenever using isoniazid to prevent peripheral neuropathy.

< 5 years	12.5mg daily
≥ 5 years	25mg daily

- Review monthly while on TPT: check for TB symptoms and side effects. Adjust dose according to weight, if needed. If peripheral neuropathy develops while on TPT ↪ 66. Advise to avoid alcohol/smoking.
- Explain possible side effects to patient: low appetite, nausea, abdominal discomfort, fatigue/weakness, dark urine, pale stools.
- If sudden new vomiting, upper abdominal pain, jaundice, hives, wheeze, difficulty breathing, BP < 90/60 or dizziness/collapse, stop TPT, refer and report ADR².
- If TB symptoms develop: send 1 sputum sample for TB NAAT ↪ 92.
- If patient interrupts TPT:
 - Explore reasons for treatment interruption, address individual concerns. Educate on the importance of adherence and provide adherence support ↪ 173.
 - Screen clinically for TB symptoms:
 - If symptoms of TB are present, check for TB ↪ 92.
 - If no symptoms TB, continue treatment including missed doses.
 - If individual interrupts for the second time, avoid restarting treatment.

Once TPT complete, decide on treatment outcome

- If completes full duration of TPT, document as **treatment completed**.
- If on 3HP, 3RH or 4R: if interrupts treatment for ≥ 4 weeks, document as **lost to follow-up**.
- If on 6H or 12H: if interrupts treatment for 3 consecutive months, document as **lost to follow-up**.
- If stops TPT due to serious adverse event or developed TB, document as **treatment stopped**.
- If died during TPT, document as **died**.

¹Rifampicin decreases levels of protease inhibitors (lopinavir/atazanavir/ritonavir), efavirenz and nevirapine: use instead **12H**. If on dolutegravir: offer **12H** or increase **dolutegravir** dose to 50mg 12 hourly. Continue until 2 weeks after TPT completed. Discuss if unsure. ²Email or fax adverse drug reaction (ADR) form to adr@sahpra.org.za or (012) 842 7609/10.